

RWOB Flight Plan to Sobriety by HALO

Recovery Without Borders · Human Advocacy & Liberative Operations

An executive brief explaining the WHMIS colour standards system, its mechanism, its rationale, and how it produces a user-input-specific flight plan to sobriety — drawn from 34 cross-referenced source documents.

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THE PROBLEM

The Addiction Services Industry Has No Standardized Language for Matching People to the Right Help

Right now, when someone in crisis contacts Ontario's addiction services, they get routed by **geography, waitlist availability, or plain-language need categories** — none of which account for the emotional, neurochemical, or archetypical profile of their condition. The result: people land in programs that may not match what's actually destabilizing them.

Simultaneously, the workers delivering those services carry **29 documented occupational stressors across 4 domains** (psychological, neurological, relational, behavioural) that the current WHMIS system does not name. There is no hazard category for compassion fatigue, vicarious trauma, or moral injury — the exact conditions that cause a 20% annual turnover rate and \$456,500 in burnout exposure per 20-person team.

Ontario's post-discharge support covers **approximately 4 weeks**. Post-Acute Withdrawal Syndrome (PAWS) lasts **6 to 24 months**. The current system covers roughly **5% of the actual neurological risk window**. Clients are released at Days 21–30 precisely as PAWS peaks — the "discharge cliff" (Gorski & Miller, 1986) — the single most consistent failure point in the system.

There is no shared vocabulary between the clinical world (CENAPS, PAWS, Marlatt), the regulatory world (WHMIS), and the client's own lived experience. Three systems, three languages, no bridge.

A Single Colour-Coded Language That Routes Clients, Protects Workers, and Measures Recovery — Using Symbols 38 Years of Canadian Workers Already Carry

The RWOB WHMIS Colour Standards Synthesis (by HALO Antifragile AI) locks the complete logic chain proving that **the colours that keep a worker safe from hazardous materials are the same colours that can describe the hazardous conditions within a person**. This is not a metaphor — it's an operational matching system tested against composite clinical cases, validated against peer-reviewed relapse prevention literature, and cross-referenced across 34 source documents.

Two specific regulatory amendments anchor everything: **GREEN** — a new WHMIS hazard category for psychological and emotional hazards (cumulative stress, vicarious trauma, compassion fatigue, moral injury). **BLACK (Expanded)** — formal recognition of the psychological shadow dimension alongside its existing biohazard/warning classification: the long-term chronic weight of hazardous clinical work.

Once the regulatory system names these hazards, support infrastructure can be built to match the burden — and the same colour vocabulary that protects the worker also routes the client to the right treatment.

What the Synthesis Does — The Mechanism

The synthesis is a **16-section reference document** that chains together every layer of the system into one auditable, cross-referenced logic path. It is not a pitch deck or a summary — it is the locked proof that the entire framework is internally consistent. Here is what each layer does and how they connect:

1. Seven Colour Domains — The Vocabulary

Five WHMIS colours currently in force (RED, BLUE, YELLOW, WHITE, BLACK), one proposed (GREEN — psychological hazard), one internal-only (GOLD — elder mastery, age 55+). Each colour carries four simultaneous meanings: a **WHMIS safety classification**, an **emotional/archetypical attribute**, a **neurochemical target** (D.O.S.E. — Dopamine, Oxytocin, Serotonin, Endorphins), and a set of **clinical stressors** it addresses.

Five of these are **identity Hues** — computed as $\text{Hue} = \text{argmax}(W, B, G, Bl, Go)$, a weighted read of curiosity and inclination over time. Two (RED, YELLOW) are **response energies** — certifiable work tiers every participant can train into and out of, never an identity anyone is assigned.

2. Dual-Track Stressor Architecture — Who It Serves

The system operates on **two completely separate tracks feeding the same equation from opposite directions**:

Track A (Client / G.A.P.S.) identifies what destabilizes the person seeking help. Built against CENAPS (Gorski 2013), PAWS neuroscience, Marlatt & Donovan (2005), and HALT. This track maps the client's symptoms through the 7 PAWS hurdles (cognitive fog, emotional overreaction/numbness, sleep disruption, stress sensitivity, cravings, anhedonia, identity confusion) and the 3 CENAPS relapse stages to specific WHMIS colour assignments.

Track B (Worker / LRP) identifies what destabilizes the person delivering help. Documents 29 occupational stressors across 4 domains — burnout, compassion fatigue, vicarious trauma, cortisol dominance, boundary blurring, moral injury, and 23 more — each mapped to a specific LRP modality and source chapter. Scored 100% at triOS College.

Both tracks feed the same Human Value Function:

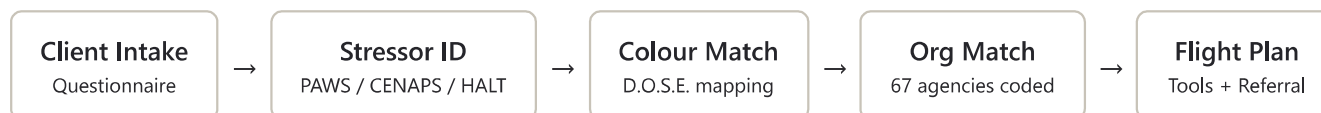
$$W = (R + B + N + V + M + Gv + Fe + Fm) \times Cs$$

9 variables, each scored 1–10. Cs (Secure Connection): Disrupted = 0.8, Developing = 1.0, Secure = 1.2

The 8 intersection points — where both nervous systems activate simultaneously in the treatment room (dysregulated systems, shame meets rescue, grief meets vicarious trauma, identity crisis on both sides, craving meets rescue, social pressure meets boundary fatigue, structural drift mirrors plan abandonment, relapse meets moral injury) — are the critical cross-reference moments the synthesis documents.

3. The Matching Engine — How Colours Route Clients to Services

This is the operational core. The synthesis documents the complete **intake-to-service routing pipeline**:



The questionnaire identifies which neurochemical systems are depleted. The colour system tells you which organizations specialize in restoring them. **Side A** maps the client's stressors to WHMIS colour attributes. **Side B** maps each organization's treatment modalities to those same colours. The match happens in the middle — a client whose stressor profile resolves to BLUE + BLACK gets routed to organizations also coded BLUE + BLACK.

67 Ontario addiction and recovery organizations across 7 regions have been pre-coded to the five WHMIS colours currently in force. Many carry blends (e.g., BLUE + BLACK for concurrent substance use and mental health in residential settings).

4. G.A.P.S. — The Client-Facing Translation Layer

Gratitude, Acceptance, Presence, Surrender. This is the psychoeducational framework that gives the client something to hold onto between treatment encounters — and when the worker is not in the room.

Each G.A.P.S. capacity targets a specific neurochemical system and addresses specific PAWS hurdles:

CAPACITY	NEUROCHEMICAL	WHAT IT INTERRUPTS	WHAT IT DOES
Gratitude	Serotonin / Dopamine	Anhedonia, emotional relapse	Redirects from threat-scanning to resource-noticing
Acceptance	Oxytocin	Identity confusion, unprocessed grief	Fish Bowl Method — observing without being controlled
Presence	Endorphins	Stress sensitivity, cravings	Creates the gap between stimulus and response
Surrender	Oxytocin / Serotonin	Social isolation, identity erosion	Releases the need to control the outcome

G.A.P.S. maps directly onto **Motivational Interviewing's four guiding principles** (Partnership → Gratitude, Acceptance → Fish Bowl, Compassion → Sacred Pause, Empowerment → Surrender). It is MI's missing self-use toolkit — MI opens the door, G.A.P.S. gives the client something to hold when the worker is not in the room.

The synthesis documents **13 specific client-use tools** (HALT Check, Sacred Pause, Fish Bowl Method, Snap Practice, Connection Text, Micro-Win, Relapse Reframe, Somatic Check-In, One-Breath Reset, 5-4-3-2-1 Ground, Name It to Tame It, Craving Analysis, 72-Hour Micro-Plan) — each mapped to its G.A.P.S. capacity, its D.O.S.E. target, and the specific stressors it addresses.

5. The Operational Pipeline — From Crisis to Sovereignty

Five sequential components, same colour vocabulary running through all of them:

PHASE	COMPONENT	DURATION	WHAT IT DOES
1	LLJ — Lifeguard's Life Jacket	90 minutes	Crisis stabilization and triage. DEEDSPROOF protocol. Entry point.
2	HAK — Home Anchor Kit	Ongoing	Private recovery tools. Self-directed bridge between treatment encounters.
3	IDW — In Deeper Waters	21 days	Structured family rebuilding. Children, partners, family systems.
4	LRP — Lighthouse Resilience	12-month cycle	Worker-facing burnout prevention. Three tiers. Runs in parallel.
5	GSRT — Global Stewardship	Ongoing	Purpose trust converting stabilized individuals into dignified workforce.

G.A.P.S. stabilizes the person. LRP protects the worker stabilizing them. GSRT gives both a destination that produces measurable value. The discharge cliff is replaced by a **Four-Book developmental continuum**: DEEDSPROOF (Day 1, WHITE) → SLOW SOAK (Year 1, GREEN/MUSCLE) → FISHING HOLE (Year 2, BLUE/HEART) → HMA 3-DAY (Year 3+, BLACK/ANCHOR).

6. The Economics — Why It Rationally Holds

Two mathematical models run in parallel, reported side by side, **never combined into a single number**:

LRP Cost Model (dollars, institutional): Current Cost = $(T+S+P+H+C) \times n$. For a 20-person node: \$306,000 current cost, \$76,500 annual savings, \$9,238 programme cost, **\$8.28 ROI per \$1 invested**, 44.1-day payback. Conservative floor ($\frac{2}{3}$ inputs): \$5.52 per \$1. Laura individual worker proof ($n=1$): identical 8.28:1 ratio — not an artefact of aggregation. Quarter-value stress test (Daniel Merrick, every input at 25% of published benchmarks): still returns 1.59:1 at 3 years.

W Human Value Function (Sovereign Units, individual): Marcus Stillwater pipeline: W 11.2 at intake → 48.0 at MASH discharge → 60.5 → 67.9 → 76.8 → 80.4 (GOLD derived). Total: +69.2 points, 618% increase. Laura: W 25.6 ($Cs=0.8$) → 74.4 ($Cs=1.2$), +48.8 units, 191% increase.

Current status: All W scores are modelled from framework logic and composite case documentation, not administered instruments. No pilot cohort has run. The synthesis names this directly — Section 8 documents seven identified holes and what pilot data would change.

7. The Gap Audit — What It Does Not Do

The synthesis includes a cross-document gap audit identifying **seven specific holes**: cannabis ambivalence (no guide addresses secondary-substance ambivalence), shame as a distinct interruption target (appears everywhere, never has its own protocol), direct social pressure scripting (no pre-rehearsed response for when someone offers a substance), structural drift detection (the most interruptible relapse point with no dedicated tool), anger as distinct from shame (different mechanisms, different responses needed), PAWS timeline in the portable guide (the most portable document contains no timeline orientation), and child conflict de-escalation (no in-the-moment tool when both parent and child are dysregulated).

This section exists because a system that hides its gaps is asking to be trusted on faith. This one documents them and names what would close them.

THE CONCLUSION — HOW IT PRODUCES A USER-SPECIFIC FLIGHT PLAN

From a Person's Own Stressors to a Named Path Through Recovery

Here is the complete chain, from input to output, for any individual who enters the system:

Step 1 — Stressor Identification. The intake questionnaire identifies the person's active stressors using validated clinical frameworks: the 7 PAWS hurdles, 3 CENAPS relapse stages, Marlatt's high-risk taxonomy, and HALT states. This is not a general wellness screening — it targets the specific neurological and psychological destabilizers that drive relapse.

Step 2 — Colour Assignment. Each identified stressor maps to a WHMIS colour through the D.O.S.E. neurochemical framework. Cognitive fog (PAWS H1) maps to WHITE (Serotonin — baseline regulation). Emotional overreaction (PAWS H2) maps to BLUE (Oxytocin — connection & depth). Stress sensitivity (PAWS H4) maps to YELLOW (Endorphins — action & recovery). Identity confusion (PAWS H7) maps to BLACK (all

four neurochemicals — full activation). The person's stressor profile produces a colour blend — their specific signature.

Step 3 — Organization Match. That colour blend is compared against the pre-coded profiles of 67 Ontario organizations. A person resolving to BLUE + BLACK gets routed to agencies whose clinical modalities address the specific conditions that colour combination represents — not the closest geographically, not the one with the shortest waitlist, but the one whose treatment specialties match what's actually destabilizing them.

Step 4 — G.A.P.S. Tool Selection. The same stressor profile determines which of the 13 G.A.P.S. self-use tools are loaded into the person's Home Anchor Kit (HAK). Someone with dominant PAWS H5 (cravings) gets the Snap Practice and D.O.S.E. substitution. Someone with dominant PAWS H7 (identity confusion) gets Observer-Thought Distinction and Lighthouse Stance. The tools are not generic coping strategies — they are targeted neurochemical interruptions matched to the specific hurdles the intake identified.

Step 5 — W Score Baseline & Trajectory. The intake stressors populate the 9 variables of the W function (Resilience, Boundary Clarity, Neurochemical Balance, Virtue Value, Meaning Quotient, Guest Virtue, Emotional Satisfaction, Morality Alignment × Secure Connection). This produces a numeric baseline. The Four-Book pathway defines the expected trajectory — not as a prediction, but as a measurement framework that makes recovery investment visible at every checkpoint (Day 42 BRC target, Day 90 HIR target >5.0).

Step 6 — The Flight Plan. The person leaves intake with: a colour profile they understand (the same symbols from their WHMIS certification), a matched organization referral, a loaded HAK with the specific G.A.P.S. tools for their stressor profile, a 72-hour micro-plan, and a W baseline that will be re-measured at Days 14, 28, 42, D+30, and D+90. They know which PAWS hurdles are their top 3. They know which G.A.P.S. capacity addresses each one. They have a scripted response for their highest-risk social pressure scenario. There is no discharge date — there is a developmental continuum through four books that maps to the same colour progression.

What makes this a flight plan rather than a referral: A referral says "go here." A flight plan says "here is what is destabilizing you (named in a language you already know), here is the neurochemistry behind it (so you know it's your brain healing, not you failing), here is the specific tool that interrupts it (so you have something to do at 2 AM when the craving hits), here is the organization whose clinical specialty matches your specific profile (so the treatment addresses what's actually wrong), and here is how we'll measure whether it's working (so recovery becomes visible before the system decides you're 'done')." The person is not a passenger — they are navigating, with instruments.

What the Synthesis Proves

The 16-section synthesis locks the proof that this chain is internally consistent — every stressor maps to a colour, every colour maps to a neurochemical, every neurochemical maps to a clinical modality, every modality maps to a specific organization, every organization has been pre-coded, and the same equation (W) measures both sides of the therapeutic relationship from opposite directions. The chain is auditable at every link. The gap audit names the seven places it isn't yet complete. The economic model survives a quarter-value stress test. The clinical cross-reference works through a composite case (Alexia — dual-substance, co-occurring depression) symptom by symptom.

The synthesis is the evidence base. The expansion letter is the ask. The flight plan is what a person actually walks out with.

